Proposed cuts: Books 7–11

Nothing has been changed yet. Every section is marked **KEEP**, **SHRINK** or **CUT**, with how often it was asked in past papers (2015–2025 and the question bank). Anything asked twice or more is never cut.

Tell me any section you want to save, and I will keep it.

Book 7	Voiding Dysfunction	35 to 25.5 pp
Book 8	Nocturia & Male Incontinence	10 to 7.5 pp
Book 9	Andrology & Sexual Medicine	45 to 31 pp
Book 10	Testis, Penis & Urethra	36.5 to 26.25 pp
Book 11	Stones, Obstruction & Trauma	41 to 28.25 pp
Total		**167.5 to 118.5 pp**

Saves about 49 pages, roughly 4 h 54 min of reading at 10 pages an hour.

Book 7 · Voiding Dysfunction

35 pages now, 25.5 pages after

Pages are estimated from the [pN] markers (35 pages, about 62 text lines per page in this file) and rounded to 0.25; rows under a quarter page are shown as 0.25, so row figures may not add exactly to the chapter totals.

Front · Title and chapter list — 0.5 to 0.5 pages

Title page, chapter list — KEEP

0.5 pages · asked: —

Navigation; stays

Ch 102 · Surgical, Radiographic and Endoscopic Anatomy of the Male Pelvis — 2.75 to 0.75 pages

1 · Bony pelvis + 2 · Lower abdominal wall (fascia, rectus sheath, inguinal canal, peritoneal folds) — CUT

0.75 pages · asked: never

General/hernia anatomy, not voiding; Colles/Buck's extravasation belongs to urethral trauma

3 · Pelvic floor and fascia + 4 · Perineum — SHRINK

0.25 to <0.25 pages · asked: never

Keep levator ani parts and tone (PFMT basis), perineal membrane, pudendal nerve S2–S4 supply; drop endopelvic/arcus fascia detail and Young-vs-Belt perineal prostatectomy MCQ

5 · Pelvic vessels and lymphatics CUT
0.25 pages · asked: never
PLND/oncology anatomy, covered in the cancer books; nothing on voiding

6 · Pelvic nerves (table, scrotal KEY POINT, P1 NVB pointer) SHRINK
0.5 to 0.25 pages · asked: never (P1 NVB in Paper 1)
Keep nerve table and scrotal key point; drop P1 pointer text already in Paper 1 answer

7 · Pelvic viscera (rectum, bladder, UVJ, pelvic ureter) SHRINK
0.25 to <0.25 pages · asked: never
Keep bladder-neck internal sphincter (α1, continence, antegrade ejaculation); drop rectum, UVJ/Hutch, ureter course — other books

8 · Endoscopic anatomy SHRINK
0.25 to <0.25 pages · asked: never
Keep verumontanum/external sphincter KEY POINT; drop cystoscopy steps

9 · Radiographic anatomy CUT
0.25 pages · asked: never
Imaging trivia (phleboliths, CT timing, node size); not voiding dysfunction

Ch 103 · Physiology and Pharmacology of the Bladder and Urethra — 5.75 to 3.25 pages

1 · Overview (two functions, single-organ KEY POINT) KEEP
0.25 pages · asked: never
Short framing

2 · Functional anatomy (bladder neck, stroma, rhabdosphincter fibre table) SHRINK

0.5 to 0.25 pages · asked: never

Keep male bladder neck collar, EUS shape, slow/fast-twitch table; drop collagen-type percentages and lamina propria detail

3 · Urothelium: barrier and sensor (mediator table) SHRINK

0.75 to <0.25 pages · asked: OAB pathophysiology ×1 (Bank)

Keep umbrella-cell barrier, ATP:NO balance, KEY POINT; drop mediator table, knockout-mouse and Cx43 myofibroblast detail

4 · Detrusor smooth muscle physiology SHRINK

0.75 to <0.25 pages · asked: never

Keep G-protein/receptor table (M2/M3/β3) and DHIC line; drop contraction cascade, Rho kinase/PKC, BK channel, gap-junction detail

5 · Receptors and drug targets (+ P3 OAB drugs, duloxetine pointers) SHRINK

1.5 to 1 pages · asked: OAB drugs ×3, mirabegron ×1

Keep M3, M2, β3, α1A, α1D, TRPV1 rows, P3 pointers and 'not in paper' lines; drop experimental rows (P2X3/AF-219, P2X1, TRPM8, NK1/aprepitant, α2, nicotinic)

6 · Peripheral innervation (P1 pointer) KEEP

0.25 pages · asked: never (P1 neurophysiology)

Core segmental levels, decentralisation, C-fibre reflex

7 · Central control SHRINK

0.25 to <0.25 pages · asked: never

Keep PMC, L-region, PAG, cortex; drop lesion-localisation KEY POINT (repeated in Ch 109 §1)

8 · Storage and emptying phases SHRINK

0.5 to 0.25 pages · asked: never

Keep guarding reflex, micturition algorithm, Pdet = Pves − Pabd; drop detrusor-power and female Pdet discussion, urethra-to-bladder reflex

9 · Urethral closure mechanisms SHRINK

0.25 to <0.25 pages · asked: never

Keep continence KEY POINT; hammock/ISD repeated in Ch 104 §2

10 · Compliance (P3 pointer) KEEP

0.25 pages · asked: Bladder compliance ×1

Asked

11 · Pathophysiology links SHRINK

0.25 to <0.25 pages · asked: never

Keep UAB vs DUA and PVR ≥300 mL line; drop BOO/NGF and failed BPS drugs (research)

12 · Ageing + 13 · Neuromodulation and botulinum toxin mechanisms KEEP

0.25 pages · asked: Botox ×7, neuromodulation ×8

Asked; mechanisms of toxin and SNM

14 · Landmark trials (ABC, ROSETTA) SHRINK

0.5 to 0.25 pages · asked: Botox ×7 (topic)

Keep ABC; drop ROSETTA row here (kept in Ch 115 §6)

Ch 104 · Pathophysiology and Classification of Lower Urinary Tract Dysfunction — 2.5 to 2 pages

1 · Normal function: two phases — KEEP
0.25 pages · asked: LUTS classification ×1
Core

2 · Pathophysiology (storage/emptying failure, hypermobility vs ISD) — KEEP
0.5 pages · asked: LUTS classification ×1
Core definitions

3 · Functional classification (Wein) 2 × 2 table — KEEP
0.5 pages · asked: LUTS classification ×1
Core classification

4 · Urodynamics by phase and site — CUT
0.25 pages · asked: never
Duplicates Wein frame in Ch 107 §1 and the tests in Ch 107

5 · Other classifications (P2 pointer, ICS urodynamic table) — KEEP
0.5 pages · asked: LUTS classification ×1 (W24-P2)
Asked; drop only the Lapides/Bors–Comarr aside

6 · Nocturia and neurogenic dysfunction — SHRINK
0.25 to <0.25 pages · asked: never
Keep polyuria/nocturnal polyuria cut-offs; drop lesion table and DSD key point (same as Ch 109 §1)

Ch 106 · Evaluation and Management of Males With Urinary Incontinence — 2.75 to 2.25 pages

1 · Definition and determinants (DIAPPERS) — KEEP

0.25 pages · asked: never

Core definitions

2 · Types (ICS table) — KEEP

0.5 pages · asked: never

Core classification

3 · Evaluation (pad severity, UDS indications) — KEEP

0.5 pages · asked: never

Standard work-up, guideline points

4 · Urgency urinary incontinence — SHRINK

0.75 to 0.5 pages · asked: OAB non-pharm ×1; Botox/SNM (topics)

Keep toxin vs SNM vs PTNS table and dementia/β3 lines; drop algorithm and drug lines (same ladder in Ch 113 §2 and Putting-together C)

5 · Stress urinary incontinence (P2 PPI pointer, EAU table) — KEEP

0.5 pages · asked: never (P2 PPI)

Guideline table and 'not in paper' points

6 · Mixed and other types — KEEP

0.25 pages · asked: never

Short, standard management

7 · Landmark trials (MASTER) — CUT

0.25 pages · asked: never

MASTER already in the Paper 2 PPI answer

Ch 107 · Urodynamic and Videourodynamic Evaluation of the Lower Urinary Tract — 4.5 to 3.5 pages

1 · Definition and role (Wein frame, Box 107.1) KEEP
0.5 pages · asked: UDS ×1
Risk-factor box is core

2 · Preparation (antibiotic prophylaxis) SHRINK
0.25 to <0.25 pages · asked: never
Keep ICS pre-invasive work-up, prophylaxis-YES list and the bacteriuria trap; drop alternative antibiotic list and NO column

3 · Non-invasive components (P2 uroflowmetry) KEEP
0.25 pages · asked: Uroflowmetry ×2
Asked ×2

4 · Equipment, transducers and quality control SHRINK
0.5 to <0.25 pages · asked: never
Keep reference level and QC KEY POINT; drop water/air/microtip transducer comparison table

5 · Filling cystometry (DO, compliance P3) KEEP
0.5 pages · asked: Compliance ×1
Asked

6 · Leak point pressures (P1) KEEP
0.25 pages · asked: UDS ×1
ALPP/DLPP cut-offs

7 · Urethral pressure profilometry KEEP

0.25 pages · asked: never

MUCP ≤20 cut-off

8 · Pressure–flow studies (BOOI, BCI, female criteria) SHRINK

1 to 0.75 pages · asked: UDS ×1

Keep interpretation table, BOOI/BCI, Groutz and Nitti criteria; drop Blaivas–Groutz/Solomon–Greenwell/Chassagne rows, stop test, bethanechol supersensitivity test

9 · EMG and sphincter coordination KEEP

0.25 pages · asked: EMG ×1; DSD ×2

Asked

10 · Videourodynamics (P4) + 11 · Ambulatory UDS KEEP

0.25 pages · asked: Videourodynamics ×2

Asked ×2

12 · Indications by condition (AUA/SUFU, EAU) KEEP

0.25 pages · asked: UDS ×1

Guideline points

13 · Landmark trials (ValUE, Dutch, SISTEr) SHRINK

0.5 to <0.25 pages · asked: never

Keep one line: office evaluation non-inferior before SUI surgery (ValUE); drop the table

Ch 109 · Neuromuscular Dysfunction of the Lower Urinary Tract — 5 to 4.5 pages

1 · Principles and classification (lesion-level table) — KEEP

1 pages · asked: Neurogenic bladder (topic)

Core classification

2 · Lesions at or above the brainstem (stroke, dementia, PD vs MSA) — SHRINK

1 to 0.75 pages · asked: Systemic disease ×1 (topic)

Keep stroke, NPH triad, PD vs MSA table; drop cerebral palsy, brain-injury detail and PD prose repeated in the table

3 · Spinal cord disease (MS, SCI, AD, myelomeningocele) — KEEP

1.25 pages · asked: Cervical spine injury ×1; DSD ×2

Asked; AD algorithm

4 · Lesions distal to the cord (P2 systemic disease) — KEEP

0.25 pages · asked: Non-GU systemic disease ×1

Asked

5 · Specific functional conditions (P1 DSD, Fowler, PBNO) — SHRINK

0.5 to 0.25 pages · asked: DSD ×2

Keep all DSD, Hinman, PBNO, Fowler; drop rare 'other conditions' (Isaacs, myasthenia, infective myelopathy)

6 · Evaluation and risk stratification (AUA/SUFU table) — KEEP

0.5 pages · asked: never

Guideline risk table

7 · Management — KEEP

0.25 pages · asked: Neurogenic bladder (topic)

Standard ladder

8 · Follow-up — SHRINK

0.25 to 0.25 pages · asked: never

Keep AUA/SUFU and EAU schedules; drop American Paraplegia Society row

Ch 110 · Overactive Bladder — 2.5 to 2 pages

1 · Definitions and terminology (P1/P2 pointer, ICS table, OAB vs BPS) — SHRINK

0.75 to 0.5 pages · asked: OAB ×1, pathophysiology ×1

Keep ICS definitions table, OAB vs DO, MUI, nocturia lines; reduce OAB-vs-BPS table to one line

2 · Epidemiology and burden — SHRINK

0.25 to <0.25 pages · asked: never

Keep prevalence 12–29% and 92% stop drugs in 2 years; drop survey names and cost line

3 · Pathophysiology + 4 · Aetiology — KEEP

0.25 pages · asked: Pathophysiology of OAB ×1

Asked

5 · Clinical assessment (contraindications table) — KEEP

0.25 pages · asked: OAB ×1

Guideline minimum work-up

6 · Urodynamics — KEEP

0.25 pages · asked: OAB ×1

Indications, PVR–toxin risk

7 · Management (conservative, third-line table) SHRINK

0.75 to 0.5 pages · asked: Non-pharm OAB ×1; Botox ×7; SNM ×8

Keep conservative measures and third-line comparison table; drop algorithm and drug paragraph (same in Ch 113 §2 and Putting-together C)

Ch 111 · Underactive Detrusor — 3.25 to 2.75 pages

P2/P4 pointer + 1 · Definitions and symptoms KEEP

0.5 pages · asked: Detrusor underactivity ×2

Asked ×2

2 · Epidemiology SHRINK

0.25 to <0.25 pages · asked: never

Keep DHIC definition and 'woman with low flow = DUA'; drop series percentages

3 · Aetiopathogenesis (mechanism table, conditions table) SHRINK

1 to 0.5 pages · asked: DUA ×2 (causes in answer)

Keep cause list, drug causes, conditions table; drop myogenic/efferent/afferent/brain mechanism table, stiff-balloon hypothesis, animal BOO and diabetes detail

4 · Diagnosis (BCI, PIP, DECO) KEEP

0.5 pages · asked: DUA ×2

Asked; indices

5 · Management KEEP

1.25 pages · asked: DUA management ×2

Asked ×2; never cut

Ch 113 · Pharmacologic Management of LUT Storage and Emptying Failure — 2.75 to 2 pages

1 · Framework — KEEP
0.25 pages · asked: never

One line

2 · Antimuscarinics and β3-agonists (P1/P3 pointer) — KEEP
0.5 pages · asked: OAB drugs ×3; mirabegron ×1

Asked ×3

3 · Combination therapy — KEEP
0.5 pages · asked: OAB drugs ×3

Asked (SYNERGY/BESIDE in answer)

4 · Intradetrusor onabotulinumtoxinA (P1) — KEEP
0.25 pages · asked: Botox ×7

Asked ×7

5 · Nocturia: desmopressin (P3) — KEEP
0.25 pages · asked: never (P3)

Doses, hyponatraemia

6 · Increasing outlet resistance (SUI drugs) — SHRINK
0.5 to 0.25 pages · asked: never

Keep duloxetine, α-agonists, imipramine rows; drop abandoned clenbuterol/phenylephrine row

7 · Facilitating emptying — KEEP
0.25 pages · asked: DUA ×2 (topic)

Bethanechol, α-blockers

8 · Failed drugs CUT

0.25 pages · asked: never

Research/experimental agents (vitamin D3, gene therapy, rimabotulinumtoxinB, intraprostatic agents)

9 · Landmark trials SHRINK

0.5 to 0.25 pages · asked: OAB drugs ×3; Botox ×7

Keep DIGNITY and EMBARK (dose trials); drop SYNERGY/BESIDE/MILAI II rows (already in §3 table and P3 answer)

Ch 115 · Electrical Stimulation and Neuromodulation in Storage and Emptying Failure — 1.75 to 1.75 pages

P3/P4 pointer + 1 · Evolution and mechanism KEEP

0.25 pages · asked: Neuromodulation ×8

Asked ×8

2 · Sacral neuromodulation KEEP

0.5 pages · asked: Neuromodulation ×8

Asked ×8

3 · Tibial nerve stimulation KEEP

0.25 pages · asked: Neuromodulation ×8

Asked ×8

4 · Other targets + 5 · Brindley KEEP

0.25 pages · asked: Neuromodulation ×8; cervical SCI ×1

In the asked answer

6 · Landmark trials (SUmiT, InSite, ROSETTA) KEEP

0.5 pages · asked: Neuromodulation ×8

In the asked answer

★ · Putting it together: voiding dysfunction decisions — 1 to 0.25 pages

A · Functional classification as treatment map — CUT

0.25 pages · asked: never

Repeats Ch 104 §3 Wein table

B · Lesion level and urodynamics — CUT

0.5 pages · asked: never

Repeats Ch 109 §1 table; keep its ≥40 cmH_2O point, already in Ch 107 Box 107.1

C · OAB ladder — KEEP

0.25 pages · asked: Botox ×7; OAB drugs ×3

The single OAB ladder kept, as the copies in Ch 106/110 are cut

D · SCI bladder — KEEP

0.25 pages · asked: Cervical spine injury ×1

Short summary

Summary

- Total now: 35 pages.

- After cuts: 25.5 pages (save 9.5, about 27%); nothing asked ×2 or more is cut.

- Biggest cuts: Ch 103 basic science (urothelium, detrusor muscle, experimental receptor rows) −2.5; Ch 102 general pelvic anatomy (abdominal wall, vessels, radiology) −2; Ch 107 transducer table, extra female criteria and trials −1; Ch 113 failed drugs and duplicate trials −0.75; Putting-together A/B duplicates −0.75.

Book 8 · Nocturia & Male Incontinence

10 pages now, 7.5 pages after

Past questions for this book: Post-prostatectomy incontinence ×2 (S21-P2, Bank; answered in full in Paper 2 answer book), Surgical therapy of male SUI ×1 (S19-P3; Paper 3 answer book), Desmopressin ×2 (W23-P3, Bank; answered in full in Paper 3 answer book). Page sizes are from the [pN] markers (p10 is a half page).

Front matter — 0.25 to 0.25 pages

Title, key to symbols, chapter list — KEEP

0.25 pages · asked: never

Navigation and key to P2/P3 markers; tiny

Ch 112 · Nocturia — 6 to 4 pages

1 · Definition and terminology — SHRINK to 0.5

0.75 pages · asked: indirect (NPi used in Desmopressin ×2 answer)

Keep ICS 2019 definition, NUV, NPi, NUP, MVV, Ni, NBCi rows and the first-morning-void trap; drop FUSP/MNi/PNV rows and the AUA/"wake to void" asides. Reason: core definitions stay, derived indices are low yield

2 · Why nocturia matters — CUT

0.25 pages · asked: never

Epidemiology, sleep-stage physiology, falls/mortality associations; not examined

3 · Pathophysiology — production, capacity, sleep

SHRINK to 0.5

0.75 pages · asked: indirect (NP threshold in Desmopressin ×2 answer)

Keep Sleep CALM, NP/global polyuria threshold table (NPi >20–33%, NUP >90 mL/h, >40 mL/kg/24 h), normal capacity and NBCi >1.3 cut-off; drop Krimpen study and the worked NBCi example. Reason: cut-offs examinable, study detail not

4 · The bladder diary

KEEP

0.25 pages · asked: never (3-day FVC also in Paper 2 OAB answer)

EAU strong recommendation, 3 days, ICI urgency grades; core and short

5 · Evaluation — Sleep CALM work-up

SHRINK to 0.25

0.5 pages · asked: never

Keep the basic work-up (diary, urinalysis, PVR, metabolic panel, STOP-BANG, drug review) and the "urology is one of five" trap; drop the ancillary-test column (echo, natriuretic peptide, copeptin, polysomnography)

6 · Sleep disorders

SHRINK to 0.25

0.5 pages · asked: never

Keep OSA link, CPAP cuts ≈2 voids, refer to sleep clinic, STOP-BANG ≥3; drop the three-arm OSA mechanism table (ANP/oxidative-stress physiology)

7 · Comorbidities

SHRINK to 0.25

0.5 pages · asked: never

Keep list of systemic causes, "always evaluate global polyuria", EAU systemic-causes point; drop DI/primary polydipsia table, non-dipping RAAS mechanism and copeptin detail (covered in nephrology)

8 · Actions — lifestyle and behaviour KEEP

0.25 pages · asked: never

Standard first-line management (fluid timing 3–4 h before sleep, sodium, weight)

9 · Lower urinary tract dysfunction SHRINK to 0.5

0.75 pages · asked: never

Keep "isolated nocturia is not LUTS", α-blocker > 5ARI, β3/antimuscarinic small benefit, Beers caution and the EAU Male/Female guideline table; drop individual trial narratives (TURP vs microwave, tolterodine vs fesoterodine). Reason: guideline points core; BPH/OAB detail duplicated in Books 7 and 16

10 · Medications causing nocturia KEEP

0.25 pages · asked: never

Short examinable drug list (diuretics, SSRIs, CCBs, lithium, SGLT-2i, ketamine)

11 · Management algorithm, desmopressin, timed diuretics KEEP

0.75 pages · asked: Desmopressin ×2 (W23-P3, Bank)

Asked ×2; the "not in paper answer" dose table, sodium monitoring schedule and timed diuretic points all stay

12 · Landmark trials CUT

0.5 pages · asked: never (as trials)

MTOPS/VA/CombAT nocturia subanalyses duplicated in BPH book; fesoterodine and desmopressin trial rows only restate Sec 9 and the Sec 11 dose table

Ch 123 · Surgical Procedures for Sphincteric Incontinence in the Male: AUS and Perineal Slings — 3.5 to 3 pages

1 · Pathophysiology and device mechanisms — SHRINK to 0.5

0.75 pages · asked: PPI ×2; Surgical therapy male SUI ×1

Keep two-sphincter concept, device table rows for AUS, Advance/XP, ATOMS, ProACT, AUS components and the ATOMS/Virtue trap; drop the Virtue row (rare variant, not in paper answers) and passive cuff-refill/sling-physiology lines

2 · Indications and device selection — KEEP

0.5 pages · asked: PPI ×2; Surgical therapy ×1

Asked ×2; pad-weight grading, AUA/SUFU strata, requirements and contraindications

3 · The high-risk ("fragile") urethra — KEEP

0.25 pages · asked: PPI ×2 (erosion risk in answer)

Erosion risk factors and countermeasures; in the paper answer

4 · Operative technique — SHRINK to 0.5

0.75 pages · asked: Surgical therapy ×1

Keep antibiotic prophylaxis, cuff site (proximal bulbar), cuff size 4.0–4.5 cm, PRB 23 mL and pressure table, 4–6 week deactivation, tandem/transcorporal/bladder-neck cuff indications; drop the step-by-step AUS incision steps and Advance XP/ATOMS/ProACT operative steps (operative minutiae)

5 · AUS complications and management — KEEP

0.5 pages · asked: PPI ×2 (revision/erosion)

Retention, infection, erosion, atrophy, mechanical failure and the leak table; standard management

6 · Sling complications and failure KEEP

0.25 pages · asked: Surgical therapy ×1

Complication rates by device; AUS after failed sling

7 · Special circumstances KEEP

0.25 pages · asked: PPI ×2 ("also know: climacturia")

Penile prosthesis sequencing, climacturia; short

8 · Outcomes KEEP

0.25 pages · asked: PPI ×2; Surgical therapy ×1 (P2, P3 in full)

Already only a pointer plus 2 lines the papers miss

★ Putting it together — 0.25 to 0.25 pages

A · Nocturia: three diary questions; B · PPI KEEP

0.25 pages · asked: Desmopressin ×2; PPI ×2

One-page synthesis of both asked topics; high-yield last-look

Total now: 10 pages (Ch 112 = 6, Ch 123 = 3.5, front matter and synthesis = 0.5).

After cuts: 7.5 pages (save 2.5, 25%); both asked ×2 topics (desmopressin, post-prostatectomy incontinence) are left whole.

Biggest cuts: Ch 112 Sec 12 landmark trials (−0.5) and Sec 2 epidemiology (−0.25); physiology and ancillary detail shrunk in Ch 112 Secs 1, 3, 5, 6, 7 and 9 (−1.25); operative steps and the Virtue row in Ch 123 Secs 1 and 4 (−0.5).

Book 9 · Andrology & Sexual Medicine

45 pages now, 31 pages after

Sizes come from the [pN] markers. Each line is weighted by how many lines are on its page (about 62 per page here). The book prints 46 pages, but p46 holds only 8 lines, so the content adds up to 45 pages. Sections smaller than 0.25 page are grouped so that rounding to 0.25 stays honest. "Asked" counts come from pq_B09. "Papers Pn" means a pointer to a paper answer the reader already reads in full.

Front matter (title, contents) — 0.5 to 0.5 pages

Title, chapter list — KEEP

0.5 pages · asked: never

Navigation only

Ch 66 · Surgical, Radiographic and Endoscopic Anatomy of the Male Reproductive System — 3.25 to 1.0 pages

1 · Scrotum — SHRINK to 0.25

0.5 pages · asked: never (lymph drainage is in Papers P1)

Keep the layer to continuity table and the scrotal-vs-testicular lymph key point. Drop the anterior/posterior wall vessels and nerves.

2 · Testis — SHRINK to 0.5

1.25 pages · asked: never directly (varicocele is in Papers P1; biopsy site supports retrieval ×7)

Keep the 3-artery table, the surgical-rules key point (biopsy site, post-vasectomy artery), the vein groups and the varicocele pointer with high-vs-inguinal ligation. Drop structure counts (lobules, tubules), the lymphatics/"trifecta" orchialgia nerves and the imaging modality table.

3–4 · Epididymis, vas deferens — CUT

0.25 pages · asked: never

Duplicated, with more depth, in Ch 67 §4–5. The embryological remnants are trivia.

5 · Seminal vesicles and ejaculatory ducts — KEEP

0.25 pages · asked: EDO ×1

TRUS cut-off (AP >16 mm / >6 mL) and the EDO imaging key point

6–7 · Prostate, urethra — CUT

0.25 pages · asked: never in B09 (these are Papers P1 pointers that belong to other books)

Duplicated in the paper answers. TRUS volume and PI-RADS belong to the prostate books.

8 · Penis — CUT

0.75 pages · asked: never directly (physiology of erection ×1 is covered in Ch 71)

Duplicated by Ch 71 §2 (corpora, tunica, arteries, veins, veno-occlusion) and Ch 72 §5 (duplex thresholds, which are given more fully there)

Ch 67 · Male Reproductive Physiology — 4.25 to 2.25 pages

1 · Hypothalamic–pituitary–gonadal axis

SHRINK to 0.75

1.5 pages · asked: Spermatogenesis ×2 (axis), andropause ×3 (ageing)

Keep GnRH pulsatility (agonist castration), Kallmann, hyperprolactinaemia, the LH/FSH table, the two-loop key point (FSH/inhibin B) and ageing changes. Drop the peptide-vs-steroid table, the GnRH modulator table, the fetal-cell product table, SRY/DAX1 and puberty triggers.

2 · Testis

SHRINK to 0.25

0.5 pages · asked: Spermatogenesis ×2, reproductive immunology ×1 (Papers P1/P3)

Keep the Sertoli/blood–testis barrier pointer, the inhibin B line and SHBG/albumin/free T binding (bioavailable T). Drop the StAR/enzyme steroidogenesis steps, the testosterone-peaks table and Leydig cell counts.

3 · Spermatogenesis

KEEP

0.75 pages · asked: Spermatogenesis ×2; genetic causes ×1; maturation arrest ×1

Asked ×2. Keeps the 64-day cycle, the AZF deletion to retrieval table, maturation arrest and paternal age.

4–5 · Epididymis; vas, seminal vesicles, ejaculatory ducts

SHRINK to 0.25

0.5 pages · asked: CBAVD ×1, EDO ×1 (indirect)

Keep the epididymal maturation key point (distal anastomosis is better; >15 y vasectomy interval) and the pH <7.2 / low volume / no fructose rule. Drop transport times, the muscle:lumen ratio and the duct-sphincter detail.

6 · Spermatozoon, capacitation and fertilisation — SHRINK to 0.25

0.75 pages · asked: never

Keep globozoospermia (PLCζ, ICSI fails) and the ciliary dyskinesia vs MMAF table, which feeds genetic causes. Drop the sperm-structure table, axoneme/dynein biology, capacitation, ZP3 and mitochondrial inheritance.

7 · Ejaculation physiology † — CUT

0.25 pages · asked: Retrograde ejaculation ×1 (Papers P3 pointer)

Duplicated by Ch 74 §2 (phases) and Ch 74 §7 (RE). The RE pointer stays there.

Ch 68 · Integrated Men's Health: Androgen Deficiency, Cardiovascular Risk and Metabolic Syndrome — 2.0 to 0.75 pages

1–3 · Men's health centre, longevity gap, urologic screening — CUT

0.5 pages · asked: never

Service organisation and epidemiology. The SHIM questionnaire is covered again in Ch 72.

4 · Urologic disorders of focus — SHRINK to 0.5

0.75 pages · asked: Male menopause/andropause ×3 (Papers P4)

Keep the TD pointer, the haematocrit AUA-vs-ES cut-offs, the opioid screening line, the fertility-caution key point and the TRT contraindication/target table. Drop the ED, BPH and infertility paragraphs, which duplicate Ch 69, 71 and 72.

5–7 · Cardiovascular prevention; obesity and metabolic syndrome; GI, skin, mental health

SHRINK to 0.25

0.75 pages · asked: never

Keep the ATP III metabolic syndrome criteria and the aromatase to low LH to low T loop. Drop the statin and BP tables (cardiology), BMI bands, colorectal screening, iron-deficiency and psoriasis lines.

Ch 69 · Male Infertility — 4.0 to 3.75 pages

1 · Definition, overview, classification

KEEP

0.25 pages · asked: Oligospermia ×3; azoospermia ×3

Definition and the pre-testicular/testicular/post-testicular classification

2 · History

SHRINK to 0.5

0.75 pages · asked: Urologist's role in REI ×1 (Papers P2)

Keep the P2 pointer, female age, paternal age ≥40, the exogenous-T key point, the gonadotoxin list and the cryptorchidism laterality table with orchidopexy timing. Drop the abstinence/lubricant advice, the infection mechanism line and the torsion line.

3 · Physical examination

KEEP

0.5 pages · asked: Azoospermia ×3

Testis-size cut-offs, vasal absence to CFTR/renal US, varicocele grading

4–5 · Endocrine evaluation; semen analysis

KEEP

0.5 pages · asked: Oligospermia ×3; semen nomenclature ×1

T/E2 ratio, prolactin action level, WHO 2021 reference limits, low-volume algorithm

6–7 · Genetic testing; imaging KEEP
0.25 pages · asked: Genetic causes ×1; EDO ×1; NOA ×2
Karyotype/YMD indications, CFTR, the EDO pointer

8 · Azoospermia: OA vs NOA KEEP
0.25 pages · asked: Azoospermia ×3; NOA ×2
FSH 7.6 / 4.6 cm table and the sorting algorithm

9 · Specific causes and treatment KEEP
1.25 pages · asked: Klinefelter ×1; CBAVD ×1; antisperm antibodies/immunobead ×1+×1; oligospermia ×3; varicocele (Papers P3)
HH table and hCG/FSH doses, clomiphene/AI table, Klinefelter trap, CBAVD, ASA to ICSI

10–11 · Lifestyle; assisted reproduction KEEP
0.25 pages · asked: Oligospermia ×3
TMSC thresholds for IUI/IVF/ICSI; retrieval route by OA/NOA

Ch 70 · Surgical Management of Male Infertility — 5.0 to 3.25 pages

1 · Principles CUT
0.25 pages · asked: never
The treat-the-man ladder is conceptual and is restated in the Putting-together key point that stays

2 · Surgical anatomy KEEP
0.25 pages · asked: Sperm retrieval/biopsy ×7 (indirect)
Preserve the testicular artery after vasectomy, never do two VVs on one vas, the single epididymal tubule

3 · Diagnostic testis biopsy KEEP
0.5 pages · asked: Surgical sperm retrieval and testicular biopsy ×7 (Papers P1)
Asked ×7

4 · Vasography — SHRINK to 0.5

0.75 pages · asked: Azoospermia ×3; EDO ×1 (indirect)

Keep the indications (absolute/relative), "only at reconstruction", the vasal-fluid/vasogram to action table, indigo carmine not methylene blue, complications and TRUS vesiculography. Drop the 6-step operative algorithm (needle gauge, suture sizes).

5 · Vasovasostomy (vasectomy reversal) — SHRINK to 0.5

1.25 pages · asked: never directly (OA management under azoospermia ×3)

Keep the evaluation and prognostic signs (FSH, indurated epididymis, granuloma), the intra-operative fluid-grading table with its key point, crossed VV indication, staging varicocelectomy after reversal, and post-op follow-up/outcomes. Drop the length-gaining steps, the healthy-cut-end description, the anastomotic principles table and the multilayer microdot suture counts.

6 · Vasoepididymostomy — SHRINK to 0.25

0.5 pages · asked: never directly

Keep the indications, distal-is-better, LIVE as the standard technique, banking sperm for late shutdown and no simultaneous varicocelectomy. Drop the 5-step algorithm and the technique comparison table.

7 · TURED — KEEP

0.25 pages · asked: EDO ×1 (Papers P3)

Pointer, retrograde ejaculation dose, brief steps

8 · Sperm retrieval — KEEP

0.5 pages · asked: Surgical sperm retrieval (TESA/TESE/micro-TESE) ×7 (Papers P2)

Asked ×7. Keeps MESA vs micro-TESE, the proximal-motile paradox and the micro-TESE steps.

9–10 · Cryopreservation; varicocelectomy

SHRINK to 0.25

0.5 pages · asked: Varicocele surgery/controversies (Papers P2, P3)

Keep the cryopreservation timing, the pointer, conventional recurrence/hydrocele rates, the vasal-vein rule and the recurrence/hydrocele/atrophy key point. Drop the 7-step microsurgical algorithm, which is operative minutiae already in the paper answer.

11 · Landmark trials

KEEP

0.25 pages · asked: never (supports varicocele)

Madgar RCT (44% vs 10%) is a standard examinable figure for varicocele benefit

Ch 71 · Physiology of Penile Erection and Pathophysiology of Erectile Dysfunction — 5.75 to 3.75 pages

1 · Definition and key concepts

CUT

0.25 pages · asked: never

The AUA definition is repeated in Ch 72 §1. The NO/PDE5I dates are history.

2 · Functional anatomy

SHRINK to 0.5

0.75 pages · asked: Physiology of erection ×1

Keep the tunica layers and 5–7 o'clock weak zone, the veno-occlusion mechanism, glans/spongiosum as a low-pressure shunt, cavernous/helicine arteries and accessory pudendals at RP. Drop the venous drainage table, sinusoidal Po2 numbers and the vascular-variation key point.

3–4 · Haemodynamics; neuroanatomy

KEEP

1.0 pages · asked: Physiology of erection ×1; neurotransmitters ×1; CRE/BEP (bulbocavernosus reflex) ×1; diabetic ED ×1 (all Papers P1)

Phase pointer, the sympathetic/parasympathetic/somatic table, the three types of erection, central promoters and inhibitors

5 · Molecular mechanisms of smooth muscle tone

SHRINK to 0.5

1.0 pages · asked: Neurotransmitters ×1 / physiology ×1 (indirect)

Keep the Ca2+–calmodulin–MLCK contraction pathway, NO–cGMP–PKG, the NO-independent cAMP pathway (why alprostadil works), PDE isoforms, the second-messenger table and the detumescence key point. Drop RhoA detail beyond one line, gap junctions, the O2 switch, the adenosine priapism mouse models and the acetylcholine/atropine MCQ.

6–7 · Classification; risk factors and endothelial dysfunction

KEEP

0.5 pages · asked: Diabetic ED ×1

Classification of ED (core), MMAS, ED as sentinel (~2 y before CAD)

8 · Pathophysiology by type

SHRINK to 1.25

2.25 pages · asked: Evaluation of diabetic with ED ×1

Keep psychogenic, the neurogenic SCI table, iatrogenic/pelvic fracture, hormonal (T thresholds, prolactin), the DM paragraph and drug-induced ED (classes and alternatives only). Drop the arteriogenic risk detail and TGF-β hypoxia, the venous-leak cause list and key point (keep "venous surgery fails"), CKD/HIV/COPD associations, anticonvulsant trivia and primary-ED mechanisms.

Ch 72 · Evaluation and Management of Erectile Dysfunction — 5.25 to 4.0 pages

1 · Definition and significance — KEEP

0.25 pages · asked: Diabetic ED ×1 (DM strongest disease)

AUA definition, prevalence, CVD sentinel, risk-factor table

2 · Management principles — KEEP

0.25 pages · asked: Diabetic ED ×1 (Princeton, Papers P1)

Shared decision making, Princeton pointer

3–4 · Basic evaluation; hormonal evaluation — KEEP

1.0 pages · asked: Diabetic ED ×1; andropause ×3 (Papers P4)

Psychogenic-vs-organic table, IIEF/SHIM, labs, T 300/150 thresholds

5 · Specialised testing — SHRINK to 0.5

1.0 pages · asked: never directly

Keep the test ladder table, duplex PSV/EDV/RI thresholds and the venous-leak key point. Drop the CIS injection technique and phenylephrine dosing (this is in Ch 73), DICC parameter detail and RigiScan numbers (keep one line each).

6 · Treatment — SHRINK to 2.0

2.5 pages · asked: Tadalafil ×1; surgical management of ED ×1 (Papers P2)

Keep the algorithm, lifestyle/drug switches, the T therapy and monitoring regimens, the full PDE5I tables, nitrate rules, failure checklist, VED, MUSE, the ICI table with contraindications, combination therapy and the surgery pointer. Drop the restorative/investigational table (Li-ESWT, PRP, stem cells, botulinum; keep "Li-ESWT investigational"), topical agents and the other oral agents line.

7 · Landmark trials — CUT

0.25 pages · asked: never

REINVENT and the ED-as-CV-sentinel finding are already stated in §6 and §1

Ch 73 · Priapism — 3.25 to 2.75 pages

1 · Definition and classification — KEEP

0.75 pages · asked: Priapism diagnosis and management ×3

Asked ×3

2 · Aetiology — KEEP

0.25 pages · asked: Priapism ×3 (Papers P3)

Asked ×3

3–4 · Molecular pathophysiology; evaluation — SHRINK to 0.25

0.5 pages · asked: Priapism ×3 (evaluation, Papers P1/P3)

Keep evaluation in full (blood gas, glans soft, algorithm). Drop §3, the PDE5 down-regulation mechanism, which is research; the daily-PDE5I prophylaxis point stays in §7.

5 · Acute ischaemic priapism — KEEP

0.75 pages · asked: Priapism ×3 (Papers P2/P3)

Asked ×3

6 · Non-ischaemic priapism — KEEP

0.25 pages · asked: Priapism ×3

Asked ×3

7 · Stuttering priapism — prevention — KEEP

0.5 pages · asked: Priapism ×3

Asked ×3

8 · Landmark trials — CUT

0.25 pages · asked: never as trials

All three results are already in the text: cold irrigation 96% vs 60% (§5), early prosthesis 3% vs 40% (§5) and the PDE5I RCT missing its endpoint (§7)

Ch 74 · Disorders of Male Orgasm and Ejaculation — 4.25 to 3.5 pages

1–2 · Spectrum; physiology of ejaculation — KEEP

0.25 pages · asked: Retrograde ejaculation ×1; PE ×2 (5-HT basis)

This is now the only ejaculation-physiology section (Ch 67 §7 is cut)

3 · PE — definition and classification — KEEP

1.0 pages · asked: Premature ejaculation ×2 (Papers P1/P3)

Asked ×2

4–5 · PE — aetiology, evaluation, treatment — KEEP

1.0 pages · asked: Premature ejaculation ×2

Asked ×2. Drug table and dapoxetine vs SSRI.

6 · Delayed ejaculation, anejaculation, anorgasmia — SHRINK to 0.75

1.25 pages · asked: never directly (supports RE ×1 and EDO ×1)

Keep the DE definition, the cause table, RPLND dry ejaculation, the orgasm/aspermia algorithm and the EDO vs RE vs emission-failure table. Drop the off-label drug table (cabergoline, reboxetine, cyproheptadine, oxytocin etc.) and keep one line: "off-label, ~40%, refer psychosexual".

7–8 · Retrograde ejaculation; sperm retrieval in SCI — KEEP

0.25 pages · asked: Retrograde ejaculation ×1 (Papers P3); sperm retrieval ×7

Post-ejaculate urine cut-off, PVS to EEJ to surgical algorithm, autonomic dysreflexia

9 · Other disorders — SHRINK to 0.25

0.5 pages · asked: Haematospermia ×2 (Papers P1)

Keep the haematospermia row and pointer, painful ejaculation and climacturia. Drop ejaculatory anhedonia, post-SSRI dysfunction, POIS and orgasmic headache (rare, never asked).

Ch 75 · Surgery for Erectile Dysfunction — 2.75 to 2.5 pages

1–2 · Place of surgery; selection and counselling — KEEP

0.5 pages · asked: Penile prosthesis types/biofilms ×4; surgical management of ED ×1 (Papers P2/P3)

Asked ×4

3 · Surgical approaches — KEEP

0.25 pages · asked: Penile prosthesis ×4

Asked ×4

4–5 · Operative technique; intra-operative problems — SHRINK to 0.5

0.75 pages · asked: Penile prosthesis ×4 (types, not technique)

Keep dorsolateral dilation, the goalpost test, cylinder sizing (≤1 cm under), ectopic reservoir after robotic RP (key point), and crossover, crural and urethral perforation management with the no-catheter key point. Drop the 7-step three-piece algorithm, SST glans-suture detail and the modelling/high-rigidity cylinder detail (repeated in Ch 76 §6).

6 · Complications

KEEP

1.0 pages · asked: Penile prosthesis — biofilms ×4

Asked ×4: infection prophylaxis, coatings, salvage table, mechanical failure, erosion

7–9 · Special situations; post-op care; revascularisation

KEEP

0.25 pages · asked: Surgical management of ED ×1

Priapism/Peyronie implants, activation at 4–6 wk, revascularisation criteria

Ch 76 · Diagnosis and Management of Peyronie Disease — 3.25 to 2.75 pages

1–2 · Definition; anatomy and pathophysiology

KEEP

0.5 pages · asked: Peyronie management ×1

AUA definition, myths key point, TGF-β1, Dupuytren association

3 · Clinical features and phases

KEEP

0.5 pages · asked: Peyronie management ×1

Active vs stable table, which drives management

4 · Evaluation

KEEP

0.25 pages · asked: Peyronie management ×1

SPL, ICI ± duplex before invasive treatment, calcification grade

5 · Non-surgical management

KEEP

0.75 pages · asked: Peyronie management ×1 (Papers P1)

CCH protocol and criteria, the AUA position on oral agents, verapamil/interferon, traction

6 · Surgery

SHRINK to 0.75

1.0 pages · asked: Peyronie management ×1 (Papers P1)

Keep the indications, consent figures, the procedure-choice algorithm, plication vs grafting principles, graft ED predictors and prosthesis with modelling. Reduce the plication eponym table (Nesbit, Yachia, Essed–Schroeder, 16-dot, TAP) to one line each, and drop the graft suturing steps.

7 · Landmark trials

CUT

0.25 pages · asked: never as trials

The IMPRESS and interferon RCT results (34% vs 18%; 27% vs 9%) already appear in §5

★ Putting it together: andrology decisions — 1.5 to 0.25 pages

A–B · Infertile man work-up; azoospermia OA vs NOA

SHRINK to 0.25

0.5 pages · asked: Azoospermia ×3 (the topic stays in full in Ch 69 §3–8)

Keep the key point "reversal for a younger partner wanting >1 child; ICSI for an older partner or female factor" and the B table, which is a compact OA/NOA comparison. Drop list A, which duplicates Ch 69 §2–6.

C–F · ED ladder, priapism, Peyronie, PE

CUT

1.0 pages · asked: Topics are asked (priapism ×3, PE ×2), but the content here is a verbatim recap

Each block repeats an algorithm kept in full in Ch 72 §6, Ch 73 §1/§5/§7, Ch 76 §5–6 and Ch 74 §3–5. No asked content is lost.

Summary

Now 45 pages of content (printed as 46 pages; p46 holds only 8 lines). After the cuts: 31 pages, a saving of 14 pages (~31%). Nothing asked ×2 or more is cut.

Those topics lose only duplicates or operative and mechanistic detail.

Biggest cuts: Ch 71 erection physiology/pathophysiology (−2.0: molecular and venous-leak detail, duplicate definition), Ch 66 anatomy (−2.25: penis, prostate/urethra and epididymis/vas duplicates), Ch 67 physiology (−2.0: hormone biology, sperm structure, duplicate ejaculation physiology), Ch 70 reconstruction minutiae (−1.75), Ch 68 men's-health and cardiology (−1.25), Ch 72 investigational therapies and testing detail (−1.25) and the Putting-together recap (−1.25).

Book 10 · Testis, Penis & Urethra

36.5 pages now, 26.25 pages after

Page sizes come from the [pN] markers and line counts (37 printed pages), rounded to 0.25 per row. Tiny adjacent sections are grouped. "Asked" uses yield/pq_B10.txt. Where the book itself tags a Paper answer (P1–P4) that is not in the B10 tally, it is shown as "Paper Px answer".

Front matter — 0.5 to 0.5 pages

Title, chapter list, legend — KEEP

0.5 pages · asked: —

Navigation

Ch 78 · Surgical, Radiographic and Endoscopic Anatomy of the Retroperitoneum — 3.0 to 1.5 pages

1 · Definition, boundaries and organs — KEEP

0.25 pages · asked: Anatomy of retroperitoneum ×1 (W24-P1)

Primary vs secondary organs; key point

2 · Posterior abdominal wall and surface landmarks — CUT

0.25 pages · asked: never

Surface and rib anatomy; flank layers and lumbodorsal fascia are already in the P1 retroperitoneum answer

3–4 · Fasciae and spaces; GI relations — SHRINK to 0

0.25 pages · asked: never (fasciae in P1 answer)

Drop the fascial strata and spaces (duplicated in the P1 answer). Fold the 3-line GI hazard note (D2 on right hilum/Kocher, D3 between SMA and aorta, splenic vessels) into §5

5 · Great vessels — SHRINK to 0.5

1.0 pages · asked: ×1 via retroperitoneum; variants in Paper 1/3 answers

Drop the aortic branch table and the ureteric blood supply (basic anatomy). Keep the left renal vein tributaries, varicocele sidedness, IMV trap and the retroaortic/circumaortic/retrocaval variants

6 · Lymphatics — KEEP

0.25 pages · asked: ★ Lymphatic drainage testis ×2 + RPLND template ×2

Landing zones, right-to-left crossover

7–8 · Nerves; imaging anatomy — SHRINK to 0.5

1.0 pages · asked: never (beyond ×1 general anatomy)

Keep the ejaculation fibres (T12–L3 to hypogastric plexus) and the nerve-injury table. Drop the autonomic outflow summary, the lumbar plexus root table and the imaging paragraph (repeats §5)

Ch 79 · Neoplasms of the Testis — 5.75 to 5.0 pages

1 · Epidemiology and risk factors — KEEP

0.25 pages · asked: never

Core: GCNIS, UDT relative risk, contralateral risk

2 · Pathology and classification — KEEP

0.75 pages · asked: Classification ×1 (Bank)

WHO subtypes, markers/IHC table

3–4 · Presentation; diagnosis and initial management — KEEP

0.5 pages · asked: Testis-sparing ×1 (S16-P4); markers in Paper 1 answer

US, marker half-lives, orchidectomy, TSS indications

5 · Staging (TNM, S, IGCCCG) — KEEP

1.0 pages · asked: Seminoma stage I ×2

Core staging and risk tables

6 · Treatment principles — SHRINK to 0.25

0.5 pages · asked: never

Keep the seminoma vs NSGCT comparison table. Drop the GCNIS management paragraph (repeated in Ch 80 §2 testis-sparing)

7 · NSGCT management — KEEP

1.0 pages · asked: Growing teratoma and BEP in Paper 1/2 answers

CS I algorithm, residual mass, relapse

8–9 · Seminoma management; brain metastases — KEEP

0.5 pages · asked: ★ Seminoma stage I ×2 (S18-P2, Bank)

Never cut (×2)

10–11 · Sequelae/follow-up; non-germ-cell tumours — KEEP

0.25 pages · asked: Classification ×1 (covers Leydig/lymphoma)

Late effects; Leydig, Sertoli, lymphoma

12 · Landmark trials — SHRINK to 0.5

1.0 pages · asked: never

Keep IGCCCG, BEP×3 vs ×4, TE19 carboplatin and MRC 20 vs 30 Gy (seminoma ×2). Drop PVB vs BEP, VIP vs BEP, Albers, Williams (repeated in Ch 80) and MRC 2 vs 5 CT

Ch 80 · Surgery of Testicular Tumours — 3.5 to 2.25 pages

1–2 · Evaluation; radical inguinal orchidectomy, TSS, delayed orchidectomy — KEEP

0.75 pages · asked: Testis-sparing surgery ×1 (S16-P4)

Standard operation and TSS criteria

3 · RPLND terminology and indications — KEEP

0.5 pages · asked: RPLND template ×2 (Bank)

Definitions table, pre-op planning

4–5 · Templates; open RPLND technique — SHRINK to 0.25

0.5 pages · asked: Templates ×2

Keep the template boundaries and the nerve-sparing key point. Drop the incision and mobilisation steps, lumbar artery counts and post-op care (operative minutiae)

6–7 · Auxiliary procedures; decision making — CUT

0.25 pages · asked: never

Operative add-ons (nephrectomy, IVC and aortic repair). Decision making repeats Ch 79 §7

8 · Histology of residual mass — SHRINK to 0.25

0.5 pages · asked: never (content is in Ch 79 §7)

Keep the Fizazi factors and adjuvant rules. Drop the 40/45/15 table (repeats Ch 79)

9–10 · High-risk PC-RPLND; unique situations — CUT

0.25 pages · asked: never as such (seminoma residual repeats Ch 79 §8)

Salvage, desperation and redo detail. Seminoma residual algorithm is already in Ch 79 §8

11 · Outcomes, fertility, complications — KEEP
0.5 pages · asked: Chylous ascites in Paper 1 answer
Ejaculation, complication table, chyle algorithm

12 · Landmark trials — CUT
0.25 pages · asked: never
Williams repeats Ch 79; SEMPET is already in the PET algorithm

Ch 81 · Minimally Invasive Retroperitoneal Lymph Node Dissection — 1.75 to 1.25 pages

Intro box + 1 · Indications and rationale — KEEP
0.5 pages · asked: MI-RPLND ×1 (W25-P3) + Robotic RPLND ×1 (W25-P4)
Recent-advance topic; indications

2–3 · Guidelines and controversy; patient selection — KEEP
0.25 pages · asked: Same ×2
AUA/EAU positions, selection

4 · Unchanging operative principles — KEEP
0.25 pages · asked: Same ×2
Duplicate open template; 5-step frame

5–6 · Preparation, positioning, ports; operative steps — SHRINK to 0
0.25 pages · asked: Robotic steps covered in P3/P4 answer
Keep one line (high-fat meal the day before; no drain; home day 1–2). Drop ports and right/left dissection steps (operative minutiae, already in the Paper answer)

7–8 · Complications; outcomes vs open — KEEP
0.25 pages · asked: Same ×2
Vascular injury algorithm, MIS vs open

9 · Landmark trials (GETUG T93BP) CUT

0.25 pages · asked: never

Secondary finding of a chemotherapy trial; "complete RPLND matters" is already in §4

Ch 82 · Tumours of the Penis — 5.0 to 3.75 pages

1–2 · Overview; premalignant lesions (PeIN) KEEP

0.25 pages · asked: Premalignant penile lesions ×1 (Bank)

EQ, Bowen, Bowenoid

3 · Aetiology and risk factors SHRINK to 0.25

0.5 pages · asked: never

Keep phimosis, HPV types, LS, the circumcision point and the two-pathway key point. Drop the HIV, tobacco, trauma/obesity and PUVA rows and the AAP detail

4–5 · Natural history and spread; presentation and diagnosis KEEP

0.5 pages · asked: Lymphatic drainage (penile spread) ×2

Stepwise no-skip spread, biopsy, hypercalcaemia

6 · Histology (WHO 2022) SHRINK to 0.25

0.5 pages · asked: never

Keep usual/warty/basaloid/verrucous/sarcomatoid with nodal risk and the LVI/PNI predictor line. Drop the rare-subtype row and the features column

7–8 · Imaging; TNM 8th staging KEEP

0.75 pages · asked: Management implications ×1

Core staging

9 · Management of the primary tumour — KEEP
0.75 pages · asked: Management implications of penile malignancy ×1 (Bank)
Organ preservation, margins, stage table

10 · Inguinal nodes — KEEP
0.5 pages · asked: Inguinal lymph nodes ×1 (S17-P2)
Algorithm

11–12 · Radiotherapy; systemic therapy — SHRINK to 0.25
0.5 pages · asked: never
Keep the brachytherapy indication, no prophylactic groin RT, adjuvant RT for ENE and neoadjuvant TIP. Drop the EBRT fractionation, late-effect percentages and palliative detail

13–14 · Follow-up; non-squamous tumours — SHRINK to 0.25
0.5 pages · asked: never
Keep follow-up, melanoma and extramammary Paget lines. Drop BCC, sarcoma, lymphoma and metastasis rows (rare variants)

15 · Landmark trials — CUT
0.25 pages · asked: never
Kroon early vs delayed repeats Ch 84 §1; TIP trial repeats §12

Ch 84 · Inguinal Node Dissection — 2.0 to 1.75 pages

1–2 · Why the groin matters; indications and risk groups — KEEP
0.5 pages · asked: Inguinal LN ×1 (S17-P2)
NCCN vs EAU-ASCO risk table

3–4 · Staging work-up; lymphatic anatomy — KEEP

0.5 pages · asked: Inguinal LN ×1; anatomy in Paper 2 answer

DSNB, FNAC, femoral triangle

5–6 · Templates; open radical ILND — KEEP

0.5 pages · asked: Inguinal block dissection ×1 (S20-P3)

Radical/modified/superficial; steps

7–10 · PLND; VEIL; complications; follow-up — SHRINK to 0.25

0.5 pages · asked: VEIL ×1 (S19-P4)

Keep the PLND indication, VEIL and the complication list with key prevention (Camper plane, saphenous sparing). Drop the cause column, "others" row and antibiotic aside

Ch 83 · Tumours of the Urethra — 3.5 to 1.75 pages

1 · Benign urethral tumours — CUT

0.5 pages · asked: never

Rare lesions (leiomyoma, haemangioma, polyp); haemangioma repeats Ch 85 §5

2 · Anatomy, histology, lymphatics — KEEP

0.25 pages · asked: ★ Urethral anatomy ×3

Segments, epithelium, nodal basins

3 · Staging (AJCC 8th) — KEEP

0.25 pages · asked: never

Core staging table

4 · Male primary urethral cancer — SHRINK to 0.25

0.5 pages · asked: never

Keep the risk factors, distal surgery with 5-mm margin and the bulbar multimodal rule. Drop the RT and inguinal paragraphs

5 · Prostatic urethral cancer — SHRINK to 0.25

0.5 pages · asked: never

Keep the mucosal/ductal/stromal treatment table and the "stroma breached?" key point. Drop the prose

6 · Female urethral cancer — SHRINK to 0.25

0.5 pages · asked: never

Keep site to histology and distal vs proximal management. Drop the complication and recurrence percentages

7 · Multimodal therapy — SHRINK to 0.25

0.5 pages · asked: never

Keep cisplatin neoadjuvant and chemoradiation for SCC. Drop the regimen-by-histology and NCCN stage tables

8 · Urethral recurrence after cystectomy — SHRINK to 0.25

0.5 pages · asked: never

Keep the risk factors, urethrectomy indications and female ONB criteria. Drop the surveillance detail

Ch 85 · Benign Disorders of the Penis and Urethra — 3.25 to 1.75 pages

1 · Reconstructive surgical technique — CUT

0.25 pages · asked: never

Cautery, sounds and suture minutiae

2 · Tissue transfer: grafts and flaps — KEEP

0.5 pages · asked: Grafts/BMG ×2 (W23-P1, Bank); gracilis flap ×1

Never cut (×2)

3 · Surgical anatomy (fascial layers) — KEEP

0.25 pages · asked: Penile fracture ×1 (Bank)

Buck's vs Colles spread table

4 · Lichen sclerosus — KEEP

0.25 pages · asked: LS in Paper 1 answer

Steroid regimen, malignancy

5 · Other urethral lesions — CUT

0.75 pages · asked: never

Rare variants: haemangioma, Reiter, amyloid, fistula, diverticulum, utricle

6 · Prepuce and meatus — KEEP

0.25 pages · asked: Meatal stenosis ×1 (S19-P2)

Paraphimosis, phimosis, meatal stenosis; circumcision steps are short

7 · Penile curvature — KEEP

0.5 pages · asked: Congenital curvature in Paper 1 answer

Devine–Horton, repair algorithm

8–9 · Penile transplantation; adult acquired buried penis — SHRINK to 0

0.25 pages · asked: never

Keep 3 lines (the vicious cycle and the Pariser I–V classification), folded into §6. Drop transplantation, the operative algorithm, scrotal lymphoedema and foreign-body injection

10 · Landmark trials (African circumcision) — CUT

0.25 pages · asked: never

Same fact (~60% HIV reduction) already in §6

Ch 86 · Urethral Stricture Disease — 3.5 to 3.25 pages

1 · Definitions, anatomy, aetiology KEEP
0.5 pages · asked: Aetiology/presentation/management ×2; classification ×1
Never cut (×2)

2–3 · Presentation and evaluation; decision making KEEP
0.5 pages · asked: Stricture management ×2
Algorithm

4 · DVIU KEEP
0.5 pages · asked: DVIU ×1 (W17-P3); endoscopic techniques ×1 (S19-P1)
Favourable/unfavourable table

5 · Excision and primary anastomosis KEEP
0.5 pages · asked: Surgery after failed VIU ×1
Transecting vs non-transecting

6–7 · Substitution urethroplasty; meatal stenosis KEEP
0.5 pages · asked: Perineal urethrostomy ×2; augmented ×1; submeatal ×1
Never cut (×2)

8 · PFUI KEEP
0.5 pages · asked: PFUI in Paper 3 answer
Progressive steps, pudendal table

9–10 · Vesicourethral stenosis/fistula; complications KEEP
0.25 pages · asked: Gracilis flap ×1 (interposition)
Short

11 · Landmark trials (ROBUST III) — CUT

0.25 pages · asked: Recent advances ×2, but trial covered in P1/P2 answers

Same figures (74.6% vs 26.8%) are in the Paper 1 and 2 stricture answers

Ch 87 · Surgery of the Scrotum and Seminal Vesicles — 3.5 to 2.5 pages

1 · Surgical anatomy of the scrotum — SHRINK to 0.25

0.5 pages · asked: Scrotal lymph drainage ×2 (in Ch 78)

Keep the arterial supply table and the three-trunk key point. Drop the skin trivia (40× absorption, 5α-reductase)

2–3 · Peri-operative principles; scrotal wall excision and reconstruction — KEEP

0.25 pages · asked: Primary scrotal cancer surgery ×2 (S21-P4, W23-P1)

Never cut (×2)

4 · Hydrocele — SHRINK to 0.25

0.5 pages · asked: never

Keep technique names with "best for", the recurrence key point and sclerotherapy. Drop the step-by-step column

5–6 · Spermatocele/epididymis; varicocele repair — KEEP

0.25 pages · asked: ★ Varicocele ×4 (S18-P3, Bank×3)

Never cut (×4)

7 · Vasectomy — SHRINK to 0.5

0.75 pages · asked: never

Keep counselling, AUA occlusion, PVSA algorithm and failure types. Drop the CV/MIV/NSV comparison table and its history

8–9 · Orchidectomy and prosthesis; torsion and fixation — KEEP
0.5 pages · asked: never
Core: torsion management, fixation methods

10–11 · Chronic scrotal pain; complications — KEEP
0.25 pages · asked: never
Pain algorithm (orchialgia/varicocele overlap)

12–13 · Seminal vesicles; surgical approaches — SHRINK to 0.25
0.5 pages · asked: never
Keep Zinner, the TRUS obstruction cut-offs and transurethral unroofing. Drop the SV tumour list (rare) and the approaches table (operative minutiae)

★ Putting it together — 1.25 to 1.0 pages

A–B · Testicular mass first week; GCT by stage — KEEP
0.5 pages · asked: Seminoma ×2
One-table revision

C · Penile cancer primary and groin — CUT
0.25 pages · asked: Inguinal LN ×1
Repeats the Ch 82 §10 algorithm almost verbatim

D · Anterior urethral stricture — KEEP
0.5 pages · asked: Stricture ×9 overall
Revision table

Total now: 36.5 pages (37 printed pages, rounded per row).

After the cuts: 26.25 pages, a saving of 10.25 pages (28%). Nothing asked ×2 or more is cut, and no whole chapter is dropped.

Biggest cuts: Ch 83 urethral tumours (−1.75, never asked), Ch 78 retroperitoneal anatomy (−1.5: aortic branch and lumbar plexus tables, surface/fascial anatomy already in P1), Ch 85 benign penis/urethra (−1.5: rare urethral lesions, technique minutiae, AABP), Ch 80 (−1.25) and Ch 82 (−1.25) repeated or operative detail, Ch 87 (−1.0).

Book 11 · Stones, Obstruction & Trauma

41 pages now, 28.25 pages after

Page sizes are from the [pN] markers and line counts. This print edition has about 61 lines per page, and the rows add up to exactly 41 pages. Question counts come from `yield/pq_B11.txt`: 91 questions under 44 headings. P1–P4 refer to the paper answer books.

Front matter — 0.5 to 0.5 pages

Title, chapter list, legend KEEP

0.5 pages · asked: —

Navigation; stays

Ch 93 · Management of Upper Urinary Tract Obstruction — 4.25 to 3.0 pages

1–2 · UPJO presentation; diagnosis (US/CT/renography table, T½ >40 min, Whitaker) KEEP

0.75 pages · asked: Radionuclide studies in PUJO ×2; Congenital PUJO ×1

Asked; cut-offs and protocol

3 · Indications and choice of intervention (endopyelotomy subgroup table, secondary UPJO) KEEP

0.25 pages · asked: Failed/complicated PUJO ×2; Congenital PUJO ×1

Asked; salvage logic

4 · Endopyelotomy (antegrade, retrograde, endopyeloplasty, bleeding algorithm, follow-up) — SHRINK to 0.25

0.5 pages · asked: Retrograde endopyelotomy ×1; Failed PUJO ×2

Keep principle, contraindications (>2 cm, infection), retrograde steps, 10-year success figures and the "failure + crossing vessel to pyeloplasty" line. Drop antegrade puncture detail, endopyeloplasty and the bleeding algorithm (operative minutiae)

5 · Pyeloplasty (principles, Anderson–Hynes steps, drain/stent algorithm) — KEEP

0.5 pages · asked: Laparoscopic pyeloplasty ×1; Failed pyeloplasty ×2

Asked; core operation

6–7 · Other UPJ reconstructions (flap table, ureterocalicostomy); retrocaval ureter — SHRINK to 0.25

0.5 pages · asked: Ureterocalicostomy ×2

Keep the ureterocalicostomy row, its P2 pointer (cortex <5 mm), the key point and the retrocaval P3 line. Reduce the Y-V, Culp, Scardino and Davis rows to one line of names and indications (historical flaps, never asked)

8 · Ureteral stricture: evaluation and endourology — SHRINK to 0.25

0.5 pages · asked: never

Keep causes, "exclude cancer", the <2 cm / >20% function algorithm and the incision-direction table. Drop balloon success figures, stricture rates after URS and SWL, and "cut to the light"

9 · Ureteral stricture reconstruction ladder (UU, reimplant, hitch, Boari, descensus, ileal ureter, autotransplant) KEEP

0.75 pages · asked: Renal autotransplantation ×4; Boari ×1; TUU ×1; Upper ureteric injury ×1

Highest-yield section of the chapter

10 · Ureteroenteric anastomotic stricture CUT

0.25 pages · asked: never

Diversion topic, peripheral to this book; never asked

11 · Retroperitoneal fibrosis CUT

0.25 pages · asked: never (in this book)

The P1/P3 answers cover it in full. Keep only the one-line pointer; drop the algorithm and repeated text

Ch 94 · Upper Urinary Tract Trauma — 3.75 to 3.5 pages

P2 pointer; 1 · Mechanism; 2 · Haematuria (definition, key point) KEEP

0.5 pages · asked: Renal trauma diagnosis/management ×2

Asked; definitions

3–4 · AAST 2018 scale; imaging indications KEEP

0.5 pages · asked: Grade 4 renal injury ×1; Renal trauma ×2

Grading and indications: core

5 · Imaging studies (CT phases, CT-finding table, MiGUTS, one-shot IVP, perinephric-collection HU table) SHRINK to 0.5

0.75 pages · asked: Renal trauma ×2

Keep CT phases, the CT-finding table and the one-shot IVP. Drop the MiGUTS nomogram and the urinoma/haematoma/abscess HU table (low-yield detail)

6–7 · Nonoperative management; urinary extravasation KEEP

0.25 pages · asked: Renal trauma ×2

Standard management

8–9 · Intervention (indications, vascular control, renorrhaphy); complications and follow-up KEEP

0.75 pages · asked: Renal trauma (vascular) ×2; Grade 4 ×1

Asked

10 · Ureteral injury (AAST ureter, iatrogenic sites, imaging, damage control, repair principles) KEEP

1.0 pages · asked: Iatrogenic ureteric injury ×2; Upper ureteric injury ×1

Asked

Ch 95 · Urinary Lithiasis: Etiology, Epidemiology and Pathogenesis — 8.0 to 4.75 pages

1 · Overview and epidemiology KEEP

0.25 pages · asked: Aetiopathogenesis ×3 (background)

Risk factors (obesity, insulin resistance, fluid) feed the pathogenesis answer

2 · Physicochemistry and stone formation (saturation states, Randall plaque, plaque vs plug table) — **SHRINK to 0.75**

1.0 pages · asked: Aetiopathogenesis ×3; Urinary crystals ×1; Nanobacteria ×1

Keep CP/Ksp/Kf, the saturation-state table, RSR, Randall plaque and free vs fixed particle. Drop the stone-former plaque-vs-plug table; the P2 answer covers it ("Randall plaque vs ductal plugs")

3 · Inhibitors and promoters (table) — **SHRINK to 0.25**

0.5 pages · asked: Aetiopathogenesis ×3

Keep the inhibitor/promoter list and the citrate and Tamm–Horsfall one-liners. Drop the mechanism and key-feature columns (basic science; the P2 answer has the inhibitor list)

4 · Mineral metabolism (calcium/oxalate absorption, tubular segment table, claudins, TRPV5) — **CUT**

0.5 pages · asked: never

Detailed physiology, not examined

5 · Classification (stone composition %, metabolic abnormality table) — **SHRINK to 0.5**

1.0 pages · asked: Metabolic evaluation ×4 (indirect)

Keep the stone-composition frequency table and the key point. Reduce the 12-row abnormality table to a list of names without % ranges

6 · Hypercalciuria (definition, Pak table, CYP24A1, monogenic table, PHPT, hypercalcaemia differential) — SHRINK to 0.75

1.5 pages · asked: Metabolic evaluation ×4 (indirect)

Keep the definition (>200 mg/day, >4 mg/kg/day), the Pak absorptive/renal/resorptive table, the fasting-urine/PTH key point and the PHPT lines. Drop the Dent/FHHNC/Bartter table (repeated in Ch 54 §4), CYP24A1/phosphate leak and the hypercalcaemia differential table (general medicine)

7 · Hyperoxaluria (enteric algorithm, dietary, PH pointer) — KEEP

0.25 pages · asked: Primary hyperoxaluria ×1

Asked; cut-off 40 mg/day

8 · Hyperuricosuria, hypocitraturia, hypomagnesiuria — KEEP

0.25 pages · asked: Allopurinol ×1

Cut-offs (600, 320 mg/day)

9 · Renal tubular acidosis (type 1/2/4 table, incomplete dRTA) — KEEP

0.75 pages · asked: Distal RTA ×1; RTA ×1; Proximal RTA ×1

Asked three times as separate questions

10 · Uric acid stones (determinants table, pKa, insulin-resistance algorithm) — SHRINK to 0.25

0.5 pages · asked: Uric acid stones ×1

Keep the determinants table, uricase, pKa 5.35, the radiolucency line and the key point. Drop the NHE3/ammoniagenesis algorithm (the P2 answer covers it)

11 · Cystine and struvite stones — KEEP
0.25 pages · asked: Struvite stones ×1
Asked

12 · Miscellaneous and drug-related stones (xanthine/2,8-DHA table, urate, drug table) — SHRINK to 0.25
0.5 pages · asked: never
Keep one line each: indinavir radiolucent on CT; topiramate/acetazolamide cause CaP; 2,8-DHA and xanthine radiolucent. Drop the tables (rare variants)

13 · Anatomic predisposition and pregnancy — SHRINK to 0.25
0.5 pages · asked: Stones in pregnancy ×3
Keep the pregnancy lines and the key point. Reduce the anomaly table (UPJO, horseshoe, diverticulum, MSK stone %) to one line

14 · Landmark trials (Borghi, Ettinger allopurinol) — CUT
0.25 pages · asked: Diet ×2; Allopurinol ×1 (both covered elsewhere)
Duplicate: Borghi is in Ch 99 §11 and the allopurinol result is in §8 text

Ch 96 · Acute Care of Urinary Lithiasis — 1.5 to 1.5 pages

1 · Presentation and diagnosis (imaging table) — KEEP
0.5 pages · asked: Stones in pregnancy ×3 (US/MRI)
Core imaging choice

2 · Pain management — KEEP
0.25 pages · asked: never
Guideline first line, ketorolac dose

3–4 · MET pointer; urgent intervention (AUA indications, P-CUT, algorithm) KEEP
0.25 pages · asked: Medical expulsive therapy ×2
Asked; drainage indications

5 · Special populations (pregnancy, elderly, transplant) KEEP
0.25 pages · asked: Stones in pregnancy ×3
Asked

6 · Landmark trial (SUSPEND) KEEP
0.25 pages · asked: Medical expulsive therapy ×2
Asked ×2; the key trial in the MET answer

Ch 97 · Surgical Management of Ureteral Calculi — 3.0 to 2.5 pages

P2 pointer; 1 · Background and indications (tiers, antithrombotic table) KEEP
0.5 pages · asked: Upper ureteric stones ×1
Indications and antithrombotics: core

2 · Choosing treatment KEEP
0.25 pages · asked: Upper ureteric stones ×1
Size guidance

3 · Ureteroscopes KEEP
0.25 pages · asked: Ureteroscopes and RIRS accessories ×4
Asked ×4

4 · Access (wires, sheath, dilation table) KEEP
0.5 pages · asked: RIRS accessories ×4; Ureteroscopy ± stenting ×1
Asked

5 · Lithotripsy (energy-source table, Ho:YAG vs TFL settings, heat) KEEP

0.5 pages · asked: Energy sources ×2; New lasers ×1

Asked

6–7 · After URS; SWL for ureteral calculi SHRINK to 0.25

0.5 pages · asked: SWL outcome factors ×2; Ureteroscopy ± stenting ×1

Keep the stent indications, the SWL predictors (size, impaction ring sign, HU/SSD) and the positioning and rate lines. Drop the absolute/relative contraindication table, which appears again in Ch 98 §2 and Putting-together B

8–9 · Special situations; complications KEEP

0.25 pages · asked: Stones in pregnancy ×3

Asked; complication rates

10 · Landmark trials (stentless URS, TFL vs Ho:YAG) CUT

0.25 pages · asked: Ureteroscopy ± stenting ×1; New lasers ×1

Duplicate: both take-homes are in §5 and §6 text

Ch 98 · Surgical Management of Upper Urinary Tract Calculi — 4.25 to 3.25 pages

1 · Choosing the approach (domain table, asymptomatic stones, AUA/EAU modality table, morphometry scores)

SHRINK to 0.75

1.0 pages · asked: Lower-pole calculus ×1; Bilateral staghorn ×1

Keep the asymptomatic-stone lines, the preop CT/HU/SSD lines, the AUA modality table, the EAU line and the scores pointer. Drop the stone/anatomy/patient domain table and its key point (generic)

2 · Shock wave lithotripsy (P2 pointer, contraindications)

KEEP

0.25 pages · asked: SWL outcome factors ×2; Indications/mechanism ×1; Steinstrasse ×1

Asked

3 · Ureteroscopy / RIRS (sheath, stent omission, complications table)

KEEP

0.75 pages · asked: RIRS / flexible ureteroscopy ×5

Asked ×5

4 · PCNL (indications, positioning, access, tract size, exit, complications)

KEEP

1.25 pages · asked: Mini-PCNL ×3; Tubeless PCNL ×3; Prone vs supine ×2; Bleeding ×1; Complications ×1; Bilateral staghorn ×1

Highest-yield section of the book

5 · Special anatomy (table); laparoscopic/robotic role — SHRINK to 0.25

0.5 pages · asked: Open/laparoscopic stone surgery ×1

Keep the horseshoe, calyceal diverticulum and transplant rows and the laparoscopic/robotic line. Drop the pelvic kidney, diversion and spinal deformity rows (rare variants)

6 · Landmark trials (Lower Pole I, contralateral RCT, Cochrane) — CUT

0.5 pages · asked: Lower-pole calculus ×1

Duplicate: each take-home is already in §1 and §4 text and in Putting-together B

Ch 99 · Prevention of Urinary Stone Disease — 2.25 to 1.75 pages

1–3 · Chronic condition; screening and metabolic evaluation; general measures — KEEP

0.25 pages · asked: Metabolic evaluation ×4; Recurrent stone prevention ×5; Diet ×2

Asked

4 · Hypercalciuria: thiazides; drug/dose table — KEEP

0.5 pages · asked: Recurrent stone prevention ×5; Allopurinol ×1

Doses

5–7 · Hyperoxaluria; hypocitraturia; uric acid — KEEP

0.25 pages · asked: Primary hyperoxaluria ×1; Uric acid stones ×1; Prevention ×5

Asked

8 · Cystine stones (algorithm, thiol comparison table) — SHRINK to 0.25

0.5 pages · asked: never (in this book)

Keep the algorithm (≥3 L, <250 mg/L, pH 7.0–7.5, tiopronin). Reduce the thiol table to one line: tiopronin is the thiol of choice, 300 mg twice daily

9–10 · Infection (struvite) stones; monitoring — KEEP

0.25 pages · asked: Struvite ×1

AHA cut-offs

11 · Landmark trials (Borghi ×2, Ettinger, Barcelo, ILLUMINATE-A) — SHRINK to 0.25

0.5 pages · asked: Diet ×2; Prevention ×5

Keep Borghi 1996 (fluid) and Borghi 2002 (normal calcium), which carry the diet answer. Drop Ettinger, Barcelo and ILLUMINATE-A; their results are already in the §4–6 text and the PH answer

Ch 100 · Lower Urinary Tract Calculi — 1.5 to 0.5 pages

1 · Bladder calculi (primary vs secondary table, foreign bodies) — SHRINK to 0.25

0.5 pages · asked: never

Keep the primary vs secondary table and the key point. Drop the foreign-body list

2–3 · Management options; special situations — SHRINK to 0.25

0.5 pages · asked: never

Keep the modality table and "BOO 45–79%; bladder stone with BOO is an absolute indication for BPH surgery". Drop the diversion, spinal and transplant details

4–5 · Urethral calculi; unusual sites (preputial, prostatic, seminal vesicle) — CUT

0.5 pages · asked: never

Rare variants; never asked

Ch 54 · Management of Paediatric Kidney Stone Disease — 3.5 to 1.75 pages

1 · Epidemiology (child vs adult table) — CUT

0.5 pages · asked: never

Epidemiology; never asked

2 · Diagnosis and imaging — KEEP

0.25 pages · asked: never

US first, then low-dose CT: core

3 · Metabolic evaluation (history/examination clues, normal-values table) — SHRINK to 0.25

0.5 pages · asked: never (adult metabolic evaluation ×4)

Keep "full evaluation after the first stone" and the paediatric normal-values table. Drop the history and examination clue lines

4 · Urinary abnormalities (hypercalciuria by serum calcium, PH, monogenic table) — SHRINK to 0.25

0.5 pages · asked: never (PH ×1 under Ch 95/99)

Keep the monogenic table (Dent, Bartter, FHHNC, dRTA) and the "10–30% of PH1 respond to pyridoxine" line. Drop the serum-calcium cause table

5 · Management principles (AUA 2016 paediatric table) — KEEP

0.5 pages · asked: never

Guideline points

6–8 · Ureteroscopy; SWL; PCNL (step algorithms) — SHRINK to 0.25

0.5 pages · asked: never

Keep the numbers: 8/10 Fr calibration, then a stent for ≥14 days; SWL favourable <10 mm and <1000 HU; PCNL first line >20 mm, so miniaturise. Drop the step-by-step algorithms (operative minutiae)

9 · Secondary prevention (fluid, pH targets, drug table) — SHRINK to 0.25

0.5 pages · asked: never

Keep the fluid and pH targets and one line of child doses. Drop the drug table, which repeats Ch 99 §4

10 · Landmark trial (ILLUMINATE-A) — CUT

0.25 pages · asked: never

Duplicate of Ch 99 and the P2 PH answer

Ch 125 · Genital and Lower Urinary Tract Trauma — 2.75 to 1.75 pages

1–2 · Principles; penile fracture (P2 pointer and extras, repair algorithm) — KEEP

0.25 pages · asked: never (in this book; P2 covers it in full)

Small; only extras beyond the P2 answer

3 · Penetrating, bite, amputation, zipper, strangulation — CUT

0.5 pages · asked: never

Rare; operative minutiae (microsurgical suture sizes, transport bagging)

4 · Testicular injury (AAST testis, algorithm) — SHRINK to 0.25

0.5 pages · asked: never

Keep "US sensitivity 64–75%, so explore if equivocal", haematocele and penetrating injury mean exploration, salvage >90% within 72 h. Drop the AAST testis table and the closure algorithm

5 · Genital skin loss (graft table) — CUT

0.25 pages · asked: never

Operative minutiae; Fournier is covered in the P1/P2/P4 answers

6 · Bladder injury (P2 pointer, AAST bladder, immediate-repair key point) — KEEP

0.5 pages · asked: Pelvic fracture lower-tract injury ×2; Teardrop bladder ×1

Asked

7–8 · Posterior urethral injury (PFUI); anterior urethral injury — KEEP

0.5 pages · asked: PFUDD early vs delayed ×2; Pelvic fracture LUT ×2; Bladder neck injury ×1

Asked

9 · Landmark trial (multi-institutional PFUI study) — KEEP

0.25 pages · asked: PFUDD early vs delayed ×2

Asked ×2; evidence for SP tube vs realignment

Ch 62 · Paediatric Genitourinary Trauma — 4.0 to 2.0 pages

1 · Renal injury: how the child differs (table) — KEEP

0.5 pages · asked: never

Child-specific core

2–3 · Imaging indications; imaging studies — KEEP

0.25 pages · asked: never

Mechanism-led triggers; hypotension formula

4 · Grading (1989 vs 2011 table, high-risk CT criteria) — SHRINK to 0.25

0.75 pages · asked: never (adult grade 4 ×1)

Keep the high-risk CT criteria (haematoma >2.2 cm in a child; 0–1 vs 2–3 factors). Drop the 1989/2011 table, which AAST 2018 in Ch 94 §3 supersedes

5–6 · Non-operative management; complications (bleeding table) — SHRINK to 0.25

0.5 pages · asked: never

Keep "NOM at every grade if stable", the urine-leak escalation and superselective AE >80%. Drop the persistent vs delayed bleeding table

7–9 · Operative management; follow-up; ureteral trauma — SHRINK to 0.25

0.75 pages · asked: never (adult renal trauma ×2 and ureteric injury ×2 under Ch 94)

Keep the UPJ disruption lines, scarring by grade, "avoid TUU in children" and the timing rule (<1 week repair; later, stent and then repair at 6–12 weeks). Drop the repeated renorrhaphy algorithm, the P2/P3 pointers and the repeated AAST ureter table

10 · Bladder injury (child vs adult table, AAST) — SHRINK to 0.25

0.5 pages · asked: never

Keep the child vs adult table and the bladder-neck key point. Drop the repeated AAST bladder line

11–12 · Urethral injury (AAST/Goldman table, algorithm); genital injury — SHRINK to 0.25

0.75 pages · asked: never (adult PFUDD ×2 under Ch 125)

Keep the outcomes, "RUG before catheter", the Goldman type names (one line) and the abuse key point. Drop the AAST/Goldman table, the delayed-repair algorithm (repeats Ch 125) and the repeated penis and testis AAST lines

★ Putting it together — 1.75 to 1.5 pages

A–B · Renal colic; stone operation — KEEP

0.75 pages · asked: MET ×2; RIRS ×5; Lower pole ×1

Last-day synthesis

C · Prevention — KEEP

0.25 pages · asked: Prevention ×5; Metabolic evaluation ×4

Last-day synthesis

D–E · Renal trauma; bladder and urethra — KEEP

0.5 pages · asked: Renal trauma ×2; PFUDD ×2; Pelvic fracture LUT ×2

Last-day synthesis

F–G · Ureteric defects; children — CUT

0.25 pages · asked: Autotransplant ×4 (covered in Ch 93 §9)

F repeats the Ch 93 §9 ladder table word for word; G repeats the kept Ch 54/62 key lines

Total now: 41 pages.

After: 28.25 pages, saving 12.75 pages (≈31%). Nothing asked ×2 or more is removed.

Biggest cuts: Ch 95 stone basic science (−3.25: mineral metabolism, monogenic and hypercalcaemia tables, inhibitor mechanisms, drug stones); Ch 54 paediatric stones (−1.75); Ch 62 paediatric trauma (−2.0); Ch 93 (−1.25: ureteroenteric stricture, RPF, old UPJ flaps); Ch 100 urethral and unusual calculi (−1.0); Ch 125 penile amputation, bites and skin loss (−1.0); landmark-trial tables that repeat the chapter text (Ch 95/97/98).